STRATA CRITICAL MEDICAL INC SRTA

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by gcose4

			2026	2027
Price:	5.23	EPS	nm	nm
Shares Out. (in M):	87	P/E	nm	nm
Market Cap (in $M):	455	P/FCF	26.4	15.8
Net Debt (in $M):	-31	EBIT	22	29
TEV (in $M):	424	TEV/EBIT	19.2	14.8

Description

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Strata Critical Medical is a newly formed, and underfollowed company that sits at the physical center of one of the better secular-growth stories in U.S. healthcare: getting human organs from a donor to a transplant recipient. This is the business that used to be Blade Air Mobility. In August of last year management sold the headline-grabbing, cash-burning part of the company (the urban air-taxi/passenger business, to Joby for up to $125m) and kept the unglamorous but cash-generative part: moving and clinically recovering transplant organs. A month later they bought Keystone Perfusion, one of the largest independent organ-recovery and normothermic-regional-perfusion providers in the country, and rebranded the new entity Strata Critical Medical. What's left is the only integrated, device-agnostic platform that spans the entire non-device value chain of a transplant, from surgical organ recovery to the perfusion team to the air and ground logistics and the placement coordination.

SRTA has quickly positioned itself as a leader in a high-value, structurally growing, deeply fragmented market. The stock trades at roughly 1.6x this year's sales and ~14.5x this year's EBITDA against a net-cash balance sheet, while the one true public comp trades north of 3x sales. We think there's a credible path to the stock doubling if the clinical roll-up and the perfusion inflection play out, and a level of downside protection if they don't.

Industry Overview

Every organ transplant in America is a time sensitive logistics problem. An organ becomes available at a donor hospital, and within a window measured in hours it has to be surgically recovered, sometimes perfused to keep it viable, flown or driven, often across state lines, to a transplant center, and implanted. This is a service that cannot be imported, cannot be centralized offshore, and cannot fail. Time-critical, local, and life-or-death is exactly the kind of structure that produces durable, defensible service businesses.

The demand side is one of the more reliable growth curves in healthcare. The U.S. performed a record 48,149 transplants in 2024, up 3.3% year-on-year and up 23% over five years, and the transplant community is openly targeting 50,000 by 2026. Deceased-donor transplants have set a new record in each of the past twelve consecutive years. This is a rare thing, a healthcare end-market that grows mid-single-digits almost regardless of the economy, because the binding constraint isn't demand (there are 167,000 people on the waiting list) but supply of usable organs. And the supply of usable organs is precisely what the newer technologies are unlocking.

Two structural shifts turn that steady volume growth into something better for the people who move and recover organs. The first is the shift in how organs are allocated. The system is moving away from the old local-first model toward "continuous distribution," where geography no longer draws hard boundaries. Lungs went live in March 2023, and hearts and kidneys are in development. The practical effect is that organs now travel much farther: after lung continuous distribution, the median donor-to-hospital distance nearly doubled, from 195 to 353 nautical miles. More distance means more flying and more dedicated logistics, and, tellingly, the allocation formula now literally includes "placement efficiency," meaning how easily an organ can be transported is now a formal input into who gets it. Reliable logistics has gone from an operational afterthought to a factor in the allocation math itself.

The second, and to us more interesting, shift is the rise of donation after circulatory death (DCD) and normothermic regional perfusion (NRP). DCD donors grew 23.5% in 2024 and now approach half of all procured donors. NRP is a clinically demanding technique that restores circulation to recover otherwise-marginal organs, and it went from 12.5% of DCD heart cases in 2019 to 44% in 2023, and lifts the number of usable organs per DCD donor from 2.6 to 3.3. This matters a great deal for Strata, because NRP is staff- and skill-intensive. It needs a perfusion team, cannulation, and on-site surgical expertise, and hospitals and organ-procurement organizations overwhelmingly outsource it rather than build it in-house. Every point of NRP penetration is, in effect, a new unit of demand for exactly the clinical-recovery service Strata just bought with Keystone.

Company Overview

Strata operates two segments. Logistics (the Trinity brand, the legacy business) is the air and ground transportation of organs plus placement coordination, the largest dedicated air transporter of human organs in the country, built through the 2021 acquisition of Trinity Air Medical and years of organic growth. Clinical (the Keystone brand, acquired in September of last year) is surgical organ recovery, NRP, and cardiac perfusion services. Strata is now the only provider that can offer a customer the recovery surgeon, the perfusion team, the aircraft, and the placement desk as a single integrated service, and it can do it while staying agnostic to whichever preservation device the customer prefers.

That last point is the crux of how we think about the competitive position. What strikes us is how little scale competition there is. The market is large, deeply fragmented, and lacks a clear leader; the only other player operating at genuine scale is TransMedics, itself a roughly $630m-revenue business that only turned profitable in the last two years, not a dominant incumbent. TransMedics has built a genuinely impressive vertically-integrated "Organ-as-a-Service" model around its own proprietary OCS perfusion device, its own clinical teams, and, increasingly, its own 22-aircraft captive fleet. That model, though, requires the customer to adopt its device to unlock the integrated service. That is a commercial choice on TransMedics' part rather than a barrier it has erected, and it could open its system to other preservation technologies if it decided the economics favored doing so, but for now it leaves Strata with genuinely attractive positioning for the many customers who want flexibility. Strata allows customers to use whatever preservation technology desired, whether OCS, OrganOx, cold storage, or whatever the surgeon prefers, and we'll handle the recovery, the perfusion staffing, the flight, and the coordination. In a world where the device layer is starting to see real competition (OrganOx was bought by Terumo for $1.5bn last year; new lower-cost perfusion devices are emerging), being the neutral, device-agnostic service layer looks like a sensible place to stand.

Management is the former Blade team, Will Heyburn (co-CEO/CFO) and Melissa Tomkiel (co-CEO/GC), who have in our view demonstrated

a surprisingly astute savvy for M&A and building a business with a highly attractive competitive position. Selling the passenger business at the top of the eVTOL hype and redeploying into a cash-generative, tailwind-backed medical roll-up is exactly the kind of unsentimental capital allocation we like to see. They've committed to a roll-up and they have begun at an impressive pace: a bolt-on cardiac-perfusion deal (Ohio Valley Perfusion) alongside Q1 results, then Louisville Perfusion Services for up to $20m all-cash in early June, and just weeks later the $21.5m acquisition of HLT-NRP, a heart-and-lung transplant surgical-recovery network, at a mid-single-digit pre-synergy EBITDA multiple. That is roughly $40m-plus of capital deployed into accretive clinical bolt-ons in the first half of 2026 alone, all funded from a net-cash balance sheet, against which management frames roughly $200m of cumulative capital available for deployment through FY2029.

Earnings Trajectory

The reported historicals here are messy. The FY2023 and FY2024 numbers you'll pull off a data terminal are contaminated by the passenger business that has since been sold and reclassified into discontinued operations. The clean way to look at Strata is on continuing operations, the medical business.

On that basis: FY2024 continuing-operations revenue was $147m (all Logistics, no Clinical yet) at a $3.8m adjusted EBITDA. FY2025 revenue grew to $197m, with Logistics up ~20% organically to $177m, plus $20m of Clinical from roughly three-and-a-half months of Keystone ownership, while adjusted EBITDA nearly quadrupled to $14m, a 7.1% margin. Gross margin was 20.9%. Revenue grew 34%. Adjusted EBITDA also adds back a meaningful ~$17m of stock-based comp, so this is a business that generates real cash at the operating line but is not yet GAAP-profitable.

Looking forward, as Keystone only closed in mid-September, FY2025 captured barely a quarter of the clinical business, so FY2026 gets a full year of Clinical that will show up as enormous "growth" even before any real organic expansion. Management has guided FY2026 to $260-275m of revenue and $29-33m of adjusted EBITDA, and the Q1 print validated the trajectory, with revenue of $67m up 87% (Logistics itself up 32% organically) and adjusted EBITDA of $6.4m against $0.4m a year earlier. Our own model lands at $273m of revenue and ~$30m of EBITDA this year, building to $302m and $37m next year, and toward $360m and ~$54m by FY2029 as the mix shifts toward higher-margin clinical work and the owned-aircraft fleet.

The three drivers of that trajectory:

1. **The full-year Keystone effect plus clinical organic growth.** The clinical business is riding the NRP inflection directly, and there is a continued industry mix shift towards third-party surgical recovery and NRP, where Strata is a market leader.
2. **Operating leverage and mix.** Adjusted EBITDA margin walks from 7% toward the low-to-mid teens over our forecast as fixed corporate costs get spread over a bigger base, owned-aircraft utilization improves the logistics margin, and clinical scales.
3. **The roll-up.** With a net-cash balance sheet, roughly $200m of M&A capacity to deploy through FY2029 (current net cash, the Joby proceeds, and forecast free cash flow), and $79m of NOLs to shield the taxes on acquired earnings, management is actively consolidating a highly fragmented supply base, with three accretive clinical bolt-ons in the first half of 2026 alone. Given that demonstrated cadence, we include a conservative level of M&A in our base numbers (roughly $30m of capital deployed per year at ~6.5x pre-synergy multiple, well below the pace management has actually shown).

Factors that drive Strata's undervaluation

1. **The identity change.** Twelve months ago this was an air-taxi story with a small healthcare business attached. Today it's a healthcare logistics-and-clinical company with a passenger business it no longer owns. The old Blade shareholders left; the healthcare specialists who should own this haven't fully arrived; and the reported financials (discontinued operations, a mid-year transformational acquisition, a rebrand and ticker change) are about as hard to read as a small-cap gets. When the numbers are this messy, generalist capital stays away, and that's a large part of why we think we are able to buy at a value price given the spectrum of outcomes.
2. **GAAP losses masking cash generation.** The headline is "unprofitable small-cap that burns cash," and on a GAAP basis that's not wrong. The net line is negative and FY2025 operating cash flow was outflowing as the company invested behind the Keystone integration and its owned fleet. But the adjusted-EBITDA line is positive, growing fast, and we expect $15-22m of free cash flow before discretionary growth-aircraft capex this year.
3. **It's small, new, and hard to own.** At a ~$470m market cap with barely a year of history under its new name and identity, Strata falls below the radar of most institutional mandates and screens as "too new / too small / too complicated." Most sell-side shops have discontinued coverage. This is an opportunity and should correct over time.

Where are numbers heading?

Looking by segment, we anticipate Logistics to grow at high-single-digits (below its recent ~20-30% organic pace, deliberately conservative); we annualize the clinical business into a full FY2026 and expect it to grow to the mid-teens on the NRP tailwind; and gross margin from 21% toward the mid-20s and adjusted-EBITDA margin from 7% toward 15% over four years as scale and mix do their work. That produces the following organic base:

$mm unless noted	FY24A	FY25A	FY26E	FY27E	FY28E	FY29E
Revenue	147	197	273	302	331	362
Adj. EBITDA	4	14	30	38	46	54
Adj. EBITDA margin	3%	7%	11%	12%	14%	15%

Onto that organic base we layer a conservative level of M&A. Management deployed roughly $40m-plus into accretive clinical bolt-ons in the first half of 2026 alone (Ohio Valley, Louisville Perfusion, and HLT-NRP). We assume a more conservative pace going forward, about $30m of capital per year in FY2027-FY2029, at ~6.5x pre-synergy EBITDA multiple against ~30% acquired-business margins, and we haircut deal-year contribution to a half-year. That is materially below what the company has actually shown, and it still adds roughly $23m of revenue and ~$7m of adjusted EBITDA to FY2028. Including it, our base FY2028 lands at approximately $354m of revenue and $53m of adjusted EBITDA (a ~15% margin), with net cash still comfortably positive after funding the deals.

What stands out is how little the market has to cooperate. FY2026 is essentially locked. A full year of an already-owned clinical business plus a logistics franchise growing 30% organically in Q1 gets most of the way to guidance without much help. From there the debate is about the margin path and the roll-up, and management has its targets it has set since the rebrand.

Competitive Position & Moat

The instinctive bear objection is "TransMedics can crush them." We take it seriously, but we think it overstates TransMedics' heft and misreads where Strata actually competes. TransMedics is a mid-cap that itself only reached profitability recently, not an unassailable giant, and the market it sits in is wide open. TransMedics is racing to internalize its own logistics (79% of its missions now fly on owned aircraft), but it does so for organs attached to its own device. Strata's clinical business competes primarily not against TransMedics but

against hospitals' and OPOs' own insourced resources, the make-vs-buy decision, not a vendor bake-off. Roughly 70% of cardiac perfusion is still done in-house by hospitals that struggle to justify full-time perfusionists for an uneven surgical caseload. That's the white space Strata's clinical acquisitions (Keystone, Ohio Valley, Louisville Perfusion, and now HLT-NRP) are aimed at, and it's a different, less-contested fight than head-to-head combat with TMDX's core device-attached franchise.

The moat, such as it is at this stage, is threefold: the density and reliability of a national logistics network you cannot spin up overnight; the scarce, hard-to-hire clinical talent (recovery surgeons, perfusionists) that customers would rather rent than employ and retain; and the integration itself, being the single accountable provider for the recovery, the perfusion, the flight, and the placement. SRTA has already put together an increasingly strong position in a market where the alternative for the customer is to build and maintain expensive, specialized capacity for volumes that are inherently lumpy. There is a lot of white space to grow into and we think they are well positioned to capture substantial share.

Valuation

What should a net-cash, fast-growing, tailwind-backed consolidator of a fragmented critical-healthcare market be worth once the market stops treating it as a confusing former air-taxi company?

Start with where it sits versus the only clean public comps. Strata trades at roughly 1.6x FY2026E sales and ~14.5x FY2026E EBITDA on a net-cash basis. TransMedics, the direct peer, trades at ~3.4x TTM sales and ~16x EBITDA; CareDx, an adjacent transplant-diagnostics play, at ~3.5x sales. A business growing revenue ~38% this year with EBITDA growing faster, backed by the same volume tailwind, trading at under half the peer sales multiple, is not a demanding starting point.

Company	EV/Sales	EV/EBITDA	Note
Strata (SRTA), FY26E	~1.6x	~14.5x	Net cash; 38% rev growth
TransMedics (TMDX)	~3.4x	~15.8x	Larger, profitable, device-led
CareDx (CDNA)	~3.5x	~39x	Diagnostics; reimbursement-exposed

Base case is our organic model plus what we view as a conservative bolt-on M&A layer (~$30m/yr FY27-29 at ~6.5x), at a 12x forward exit multiple on FY2029 EBITDA, below TMDX's ~16x and in line with specialty med-tech at scale. Bear assumes organic growth stalls, no M&A, and the multiple compresses toward a pure-logistics business. Bull assumes the roll-up runs faster and the NRP inflection lifts margins to the mid-teens, and the market pays a scarcity premium. The base deploys only a fraction of the ~$200m of cumulative M&A capacity through FY2029; the remainder, and any deals in the bear column, is further upside.

FY2029E exit	Bear	Base	Bull
Revenue ($mm)	340	400	430
Adj. EBITDA ($mm)	34	66	75
Adj. EBITDA margin	10%	16%	17%
Forward EV/EBITDA	8.0x	12.0x	16.0x
Implied value / share	~$3.50	~$9.85	~$14.70
Upside / (downside) vs. $5.23	(33%)	+88%	+181%
IRR (2.5-yr hold)	(15%)	+29%	+51%

Returns are price-only from the $5.23 entry, holding the FY2029 exit value discounted back over a 2.5-year horizon. Base applies a 12x forward multiple to FY2029 EBITDA including the conservative M&A layer; bear is organic-only with multiple compression toward a pure-logistics business; bull assumes a faster roll-up, mid-to-high-teens margins, and a scarcity premium.

The base case is a high-20s IRR over a 2.5-year hold; the bull case, which requires the roll-up to run faster and the market to re-rate it, is a roughly 50% annualized return; and the bear case is a loss but not a zero, because you're anchored by a net-cash balance sheet and a logistics franchise that would still be worth something to a strategic. That is the shape of return we're comfortable underwriting in a name like this: the downside is contained by the balance sheet and the upside is tied to a roll-up management has already begun to execute.

Blending the scenarios and weighting toward the base, present fair value is roughly $7.50-8.50, a ~43-63% return from $5.23, with a base-case FY2029 target near $9.85 and a genuine path to $14-plus if the consolidation thesis plays out at pace, against roughly $3.50 of value in the bear anchored by net cash and the logistics business.

When will the price move?

1. **Full-year clinical annualization on the tape.** As each 2026 quarter prints a full period of Keystone alongside 30%-ish organic logistics growth, the "confusing small-cap" narrative gives way to a fast-growing, two-segment healthcare-services company that screens properly for the first time.
2. **Roll-up execution.** Additional accretive bolt-ons, funded with net cash and NOL-shielded, compound EBITDA and validate the consolidation thesis. Ohio Valley, Louisville Perfusion, and the $21.5m HLT-NRP deal are the proof points already on the board in the first half of 2026.
3. **Margin delivery.** Adjusted EBITDA margin walking from 7% toward the low teens, in line with guidance, steadily retires the "will it ever make money" objection and pulls generalist capital off the sidelines.
4. **Sponsorship and re-rating.** It takes only a modest broadening of the shareholder base, or a single sell-side initiation that frames it as the device-agnostic pure-play on transplant volume growth, to re-rate a $470m company.

Risks & Mitigants

1. **It's new and unproven.** The clinical segment is barely a year in the portfolio, the company has one full year under its new identity, and there simply isn't the multi-cycle track record we'd normally want. Mitigant: true, and it's why this is sized as a higher-variance position rather than a core holding. But the underlying businesses aren't new. Trinity has been moving organs for years and Keystone is an established recovery/perfusion provider, and the volume tailwind driving both is about as well-documented as secular growth stories get. We're paying an early-days multiple for an early-days story; the point is the price reflects the uncertainty.
2. **GAAP losses and cash burn.** The company loses money on a reported basis and consumed cash in FY2025. Mitigant: the operating business generates positive and fast-growing adjusted EBITDA, management guides to positive free cash flow before growth capex this year, and the balance sheet is net cash, with management framing roughly $200m of cumulative deployment capacity through FY2029. This is a company investing behind growth from a position of financial strength, not one scrambling for survival, and the market is conflating the two.
3. **TransMedics.** The clear leader is vertically integrating logistics and could squeeze the third-party opportunity on device-attached organs. Mitigant: TransMedics' internalization applies to organs on its own device; Strata's core clinical fight is outsourcing-vs-insourcing at hospitals and OPOs, a larger and less-contested pool, and its device-agnostic model looks more valuable, not less, precisely as device-layer competition intensifies. Two players can win in a market this large and this

fragmented.
4. **Customer concentration.** There are only 55 OPOs in the country and Strata's revenue skews toward the larger, better-performing ones. Mitigant: concentration cuts both ways. These are deeply embedded, mission-critical relationships where reliability, not price, is the deciding factor, and the regulatory push toward consolidating volume into fewer, higher-performing, NRP-adopting OPOs concentrates spend among exactly the customer type most likely to outsource to Strata.
5. **M&A/integration risk.** The growth story leans on acquisitions (Trinity, TOPS, Keystone, Ohio Valley, Louisville Perfusion, HLT-NRP), which carry integration and key-person retention risk, and the pace has clearly picked up. Mitigant: management has, so far, integrated well and allocated capital with discipline. The passenger divestiture, the Keystone purchase at a sensible ~9x, and the HLT-NRP deal at a mid-single-digit pre-synergy multiple were all sensible.

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Catalyst

- Full-year annualization of the Clinical (Keystone) segment through 2026, delivering clean two-segment financials a generalist can finally read
- Continued NRP/DCD penetration driving third-party surgical-recovery and perfusion demand
- Accretive, NOL-shielded bolt-on M&A compounding EBITDA and validating the roll-up
- GAAP breakeven/profitability (~FY2026) unlocking a broader institutional shareholder base
- Broadening ownership and sell-side coverage framing SRTA as the device-agnostic pure-play on transplant-volume growth

Messages

Subject	Questions
Entry	07/28/2026 05:05 PM
Member	Artz0423

Thanks for the writeup, super interesting. This one has some of the ingredients I usually look for. However, your bull case seems to capitalize adjusted numbers at a premium multiple while management is paid to spend cash in order to reach those adjusted numbers.

Couple questions:

Thoughts on Co-CEO structure? I don't like it and don't think it's worked well for many companies historically.

How are you thinking about future SBC/dilution from here?

What's the risk of commercial air taking share versus private? There are already examples of this happening and it's significantly cheaper for some organs.

Why would they guide to FCF 'before discretionary growth capex' if owned-aircraft utilization is supposed to be one of their margin drivers? It's not discretionary capex if it's required to hit margin targets. I'm sensitive to this since FCF this year is $0 and it looks like every dollar of EBITDA growth from here requires cash out the door for aircraft growth and acquisitions. So is this more of a rollup than specialty healthcare logistics? If so that's fine, but then you're dependent on the exit multiple, and TMDX as a comp doesn't make sense given SRTA doesn't participate in any device economics. SRTA is not a great business, more of a 'people and planes' business. Doesn't mean the stock can't work, but 12-15x may be too generous a multiple.

How do you feel about corporate governance / management in general? I agree they've done some smart things and have shown solid M&A ability, but compensation literally pays them for spending money. No per share metrics being paid attention to, STIP and PSUs eat into cash flow, and no significant insider buying?